DIAGNOSTIC REPORT

MC-7005

DDRC agilus diagnostics

PATIENT NAME : NEETHU M **REF. DOCTOR :** SELF

NEETHU M	ACCESSTION NO : **4100YF001871**	AGE/SEX : 28 Years Female
	PATIENT ID : NEETF2106974100	DRAWN : 21/06/2025 18:32:06
	CLIENT PATIENT ID :	RECEIVED : 21/06/2025 18:34:34
	ABHA NO :	REPORTED : 22/06/2025 11:56:11

CLINICAL INFORMATION :

SRF ID : 170/EKM/2025062554
ICMR Registration No : DDRCE001

Test Report Status **Final**	Results	Biological Reference Interval	Units

MOLECULAR BIOLOGY

COVID 19 RT PCR

SPECIMEN	Nasopharyngeal / oropharyngeal Swab
N GENE	DETECTED
ORF1ab/RDRP	DETECTED
RESULT	SARS CoV-2 RNA DETECTED
FINAL REPORT	POSITIVE

Interpretation(s)
COVID 19 RT PCR-Method: Real-time PCR, This is a real-time RT-PCR test intended for the qualitative detection of nucleic acid from the 2019-nCoV in upper and lower respiratory specimens (such as nasopharyngeal or oropharyngeal swabs, sputum, lower respiratory tract aspirates, bronchoalveolar lavage, and nasopharyngeal wash/aspirate or nasal aspirate) collected from individuals who meet 2019-nCoV clinical and/or epidemiological criteria. The assay uses RNA extracted from clinical samples. Using the RNA extracted, the assay performs the RT-PCR reaction by dividing it into two assays for accurate detection of SARS-CoV-2. Each assay amplifies E gene and the COVID -19 specific target, RdRp gene, if present thus it is designed for both the screening and specific detection of 2019-nCoV.

Pathogen information: Coronaviruses are non-segmented positive-stranded RNA viruses with a roughly 30 kb genome surrounded by a protein envelope. Most coronaviruses cause diseases in their particular host species those that can infect humans through cross-species transmission have become an important threat to public health. Since December, 2019, severe acute respiratory syndrome coronavirus 2 (SARS-CoV-2) has been recognised as the causal factor in a series of severe cases of pneumonia originating in Wuhan in Hubei province, China. This disease has been named coronavirus disease 2019 (COVID-19) by WHO. Severe acute respiratory syndrome-related coronavirus (SARSr-CoV) is a species of coronavirus that infects humans, bats and certain other mammals. It is a member of the genus Betacoronavirus and subgenus sarbecoronavirus. Two strains of the virus have caused outbreaks of severe respiratory diseases in humans: SARS-CoV , which caused the 2002-2004 outbreak of severe acute respiratory syndrome (SARS), and SARS-CoV-2, which is causing the 2019–20 pandemic of coronavirus disease 2019 (COVID-19). Other strains of Sarbecovirus are only known to infect non-human species: bats are a major reservoir of many strains.

Interpretation: Positive results are indicative of the presence of SARS-CoV-2 RNA clinical correlation with patient history and other diagnostic information is necessary to determine patient infection status. Positive results do not rule out bacterial infection or co-infection with other viruses.

Negative results do not preclude SARS-CoV-2 infection and should not be used as the sole basis for patient management decisions. Negative results must be combined with clinical observations, patient history, and epidemiological information. A false negative result may occur, if an inadequate number of organisms are present in the specimen due to improper collection, transport or handling. False negative results may also occur if amplification inhibitors are present in the specimen. A single negative test result, particularly if this is from an upper respiratory tract specimen, does not exclude infection. Repeat sampling and testing of lower respiratory specimen is strongly recommended in severe or progressive disease. The repeat specimens may be considered after a gap of 2 – 4 days after the collection of the first specimen for additional testing if required.

For inconclusive results, repeat testing is recommended on a fresh specimen after three days for further confirmation

DDRC AGILUS cannot guarantee the integrity of Covid19 specimens collected or sourced from outside DDRC AGILUS collection centres .It is the responsibility of those collection points to ensure proper methods of sample collection and transportation.

****End Of Report****
Please visit www.agilusdiagnostics.com for related Test Information for this accession

KRISHNACHANDRAN
MOLECULAR GENETICIST

Dr.ASWATHY VARGHESE,
MBBS, MD(MICROBIOLOGY)
(Reg No - TCMC:50839)
CONSULTANT MICROBIOLOGIST

View Report

PERFORMED AT :
DDRC AGILUS PATHLABS LIMITED
DDRC agilus TOWER, G-131,PANAMPILLY NAGAR,
ERNAKULAM, 682036
KERALA, INDIA